



STEADY PRODUCT AND ENGINEERING

GUI and Decision-Surface Engineering Handoff

Turning events, measures, gates, summaries, and audit history into screens people can understand and act on.

**FOR SENIOR ENGINEERS, PRODUCT LEADERS, CLINICAL WORKFLOW REVIEWERS, DATA ENGINEERS,
AND SECURITY REVIEWERS**

August 28, 2026

Product and engineering direction

Does not approve clinical use or alter Steady's safety rules

Handoff map

This document starts with what Steady looks like now, compares six public product interfaces, then specifies the target role views, presentation contracts, gates, components, routes, tests, and build order.

Current state	Sections 1-3	Decision, scope, public routes, iOS screens, safety copy, and contrast
Market patterns	Sections 4-5	NeuroFlow, Blueprint, Greenspace, Osmind, Spring Health, and Eleos
Target product	Sections 6-10	Roles, navigation, presentation layers, gates, and screen wireframes
Build system	Sections 11-17	Components, tokens, charts, states, audit behavior, endpoints, web and iOS
Delivery	Sections 18-23	Build order, backlog, release criteria, tests, telemetry, and final rules

Build principle

Design from the decision backward. Build from the evidence forward.

1. Executive decision

Steady does not need a new data foundation before it can become easier to use. It needs a presentation layer that turns its existing foundation into clear decisions.

The system already records events, scores measures, runs deterministic gates, validates cited summaries, logs review actions, and preserves audit history. The current screens often expose those parts as separate facts. The next GUI should join them into one repeated pattern:

What changed? Why does it matter? What can this user do now? What will be recorded when they act?

The product direction is a role-based quiet cockpit. It should feel calm without hiding urgency, data-rich without leading with tables, and trustworthy without forcing a user to read implementation details before taking the next safe step.

The core design rule is:

Action first. Meaning second. Evidence third. Raw records last.

This does not mean hiding evidence. It means placing evidence one layer below the decision surface, with timestamps, sources, policy versions, and review status available on demand.

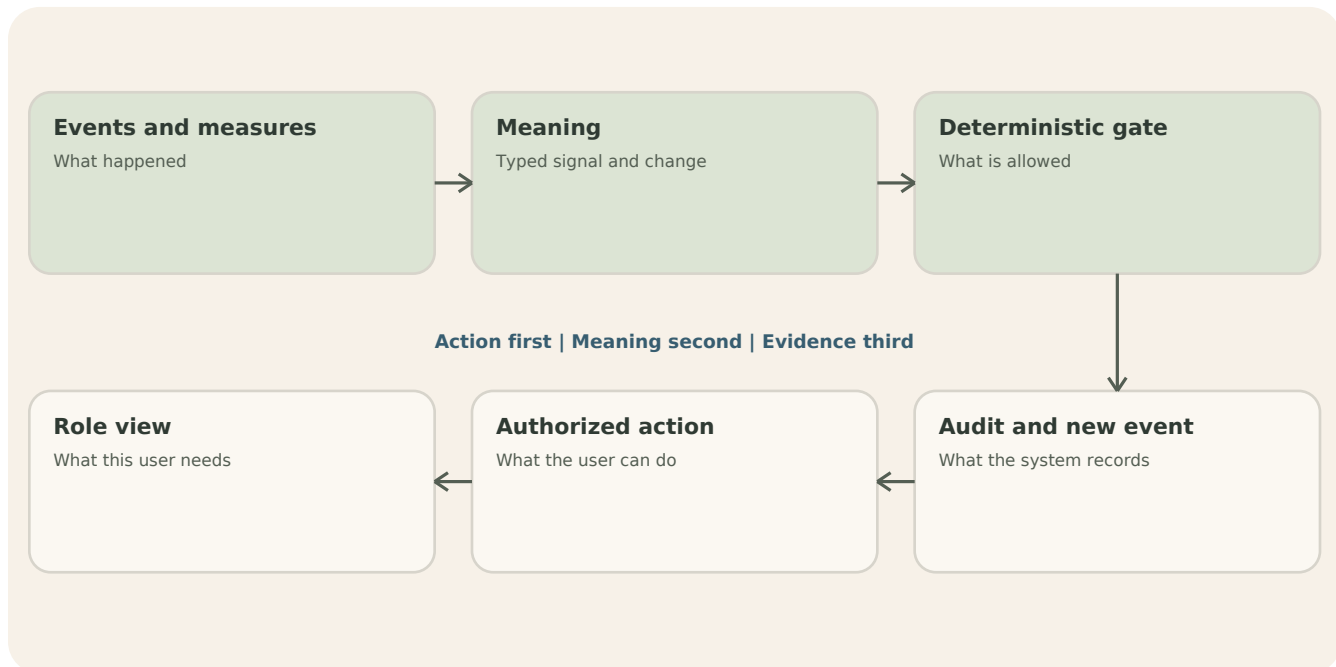
What should change

- Replace the member Home inventory with a focused Today screen containing one primary action and no more than two secondary actions.
- Replace clinician dashboards led by charts with a work queue led by priority, reason, owner, freshness, and next action.
- Give each person record a current-state header, a “since last review” summary, an action area, and a unified timeline.
- Present gates as understandable states with a reason, a safe alternative, and an explicit path forward.
- Separate deterministic decisions, human decisions, and model-generated suggestions visually and in the API contract.
- Give organization and payer users aggregate views built around outcomes, engagement, operations, and denominators. Do not reuse the clinician person view.
- Keep audit, replay, citations, and rule details available through drawers and dedicated views.

What should not change

- Deterministic safety gates remain authoritative.
- A model cannot promote itself into a governing role.
- Corrections append and supersede. They do not erase history.
- Safety stops cannot be relaxed through ordinary override controls.
- Grounding and crisis resources remain reachable even when a write, subscription, sync, or service fails.

- Tenant, role, consent, and minimum-necessary checks occur before a presentation object reaches the client.



2. Scope and evidence boundary

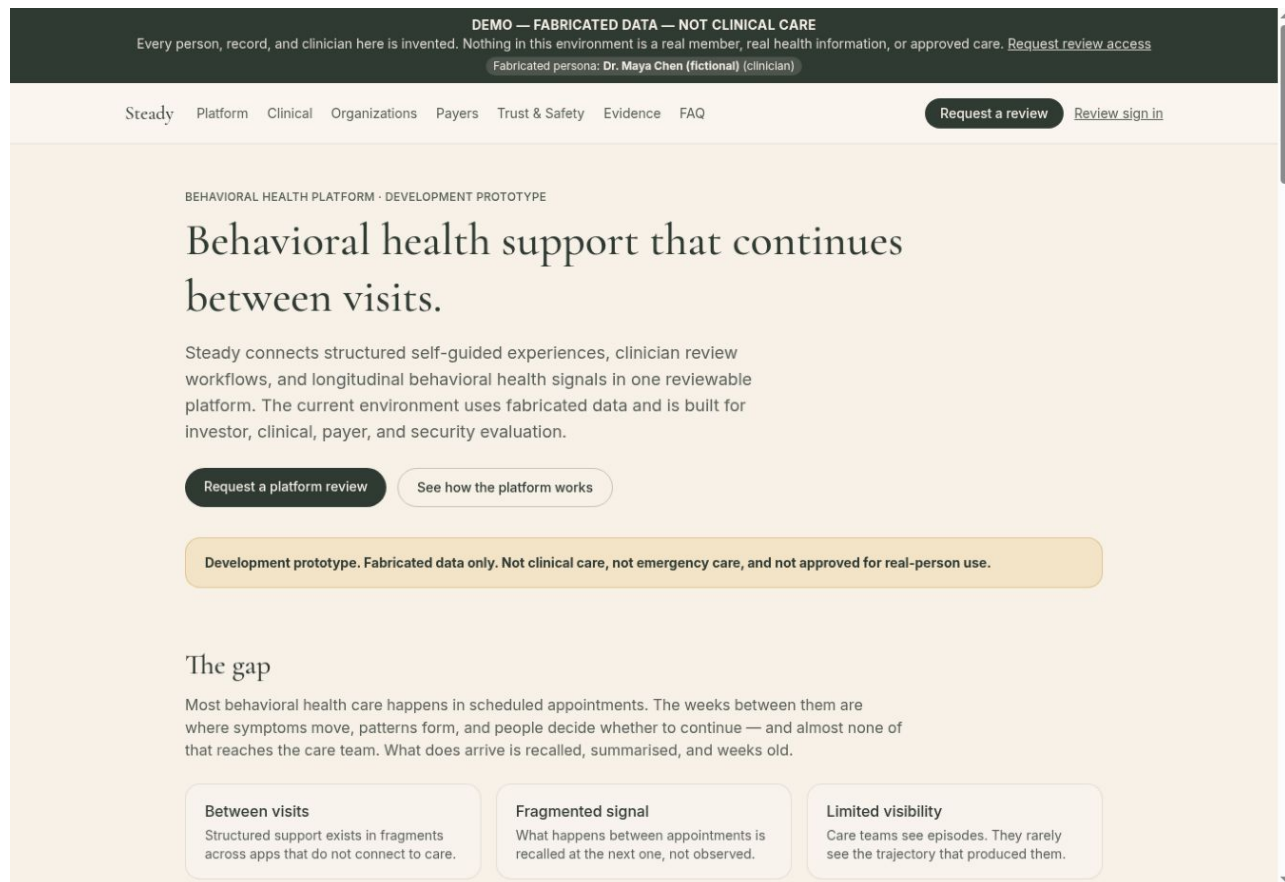
This handoff is based on four evidence sets.

- 1 The public Steady web environment was inspected route by route on August 28, 2026. Fourteen public routes were reviewed, including the institutional, trust, evidence, sign-in, and crisis surfaces.
- 2 The current Steady web repository `tprawlins-lang/Emdr` was reviewed at commit `c39447a715e92b4800889c7afd6d015e39457c8f`, dated August 27, 2026. All 53 page routes were inventoried, with close source review of the member dashboard, review gateway, clinician dashboards, caseload, person records, alert trail, autonomous console, BLS oversight, testing, and audit views.
- 3 The Steady iOS repository `tprawlins-lang/Steady-ios` was reviewed on its `main` branch at commit `f9a2645fc57135dfe1c7ee61c0e327a4ddc35a0d`, dated July 29, 2026. Every SwiftUI view and session state was inventoried.
- 4 Public product pages, help material, app listings, and product screenshots were reviewed for NeuroFlow and five comparable systems: Blueprint, Greenspace, Osmind, Spring Health, and Eleos.

The public deployment does not expose review credentials, so the locked reviewer paths were not directly observed in that deployment. Their current structure, content order, controls, states, and route relationships were confirmed from the current source. Public pages were both visually inspected and checked against source. This distinction matters because a source-confirmed layout can establish what the screen renders, but it cannot substitute for formal usability testing with each role.

Competitor findings are also bounded. Public screenshots often show selected paths and may be marketing composites. They support layout and information-pattern findings, but they do not prove every state, permission rule, or current production detail.

3. Current Steady visual system



Current Steady public home, inspected August 28, 2026. The visual language is calm and credible, but the page behaves like a long evidence document rather than an application surface.

The public site uses an ivory and linen base, dark green text, serif display headings, restrained borders, rounded cards, and muted semantic colors. Most institutional content sits in an 896-pixel column. The home route widens to 1,024 pixels. This gives the brand a composed, human tone.

The strongest current visual choices are worth keeping:

- Ivory and linen surfaces reduce glare and separate Steady from generic blue healthcare software.
- Dark ground text provides strong readability on light surfaces.
- Serif headings give the product a calm identity, while the sans-serif body font keeps dense copy readable.
- Rounded cards and generous spacing lower the emotional temperature.
- Safety messages use words as well as color.
- The iOS session player keeps pause, stop, grounding, and crisis paths within reach.

The main problem is not the brand. It is information structure.

3.1 Public route inventory

Route	Current visual form	What it asks the reader to do	Main issue
/	Long institutional landing page, cards, badges, one status table	Understand the entire platform	Too many claims and system facts compete before role selection
/platform	Narrow explanatory page	Learn the three platform layers	Sequence is described, not shown as a working interface
/clinical	Workflow copy and policy table	Judge clinical accountability	Describes actions without a visible work queue or person record
/organizations	Operating-model narrative	Consider a pilot	No aggregate operating dashboard is shown
/payers	Measurement narrative	Consider an evaluation	No denominator-led payer view is shown
/trust	Very long control and gap inventory	Review security posture	Correctly detailed, but difficult to scan and prioritize
/evidence	Evidence-status narrative	Separate method evidence from Steady evidence	Good claims discipline, limited visual structure
/faq	Long grouped question list	Resolve objections	Dense and repetitive for return visitors
/about	Position and stage narrative	Understand why Steady exists	Reads as a document, not a role entry point
/request-review	Review-path selection	Request access	Clear, but no preview of the guided route after approval
/login	Centered sign-in card	Enter the review environment	Clean and focused
/crisis	Narrow support page	Reach immediate help	Clear, compact, action led
/terms	Legal text	Review demo terms	Appropriate document form
/privacy	Legal text and one table	Review demo data handling	Appropriate document form

The public site should remain information-rich, but the home route should send each role to a short, visual preview of its actual workspace. Trust, evidence, terms, and privacy can remain document-like because their job is reference and review.

3.2 Current web application inventory

The current web repository contains 53 page routes. It already implements most of the raw material needed for a strong role-based product, but the material is distributed across long pages and overlapping consoles.

Surface group	Current routes	Current visual form	Main GUI issue
Review entry	/demo, /demo/[path], /login	Boundary notice, path cards, access code, fabricated persona cards	Clear entry sequence, but it does not preview the workspace or task the reviewer will enter
Member core	/dashboard, /check-in, /companion, /ground, /paths, /measures	Long stacked dashboard, cards, charts, libraries, forms	The dashboard mixes status, next steps, five practices, measures, paths, plan, and full module catalog
Member setup and controls	Onboarding, screening, consent, billing, account, voice, memory, session settings	Long vertical forms and repeated cards	Strong disclosure, weak sense of place and progress across the full setup sequence
Specialist entry	/clinician and /clinician/member/[id]	Alerts, review work, member table, outcome charts, long detail record	This older entry overlaps with the newer clinical console and creates two mental models

Surface group	Current routes	Current visual form	Main GUI issue
Clinical work	/clinician/clinical, /clinician/clinical/[id], /clinician/alerts/[id]	Alert list, need-ordered caseload, cited summary, timeline, override, audit	Good evidence discipline, but decisions, evidence, corrections, and audit compete in one vertical flow
Governance and validation	/clinician/autonomous, /clinician/bls, /clinician/testing, /clinician/audit	Simulation forms, engine outputs, sign-off registers, state tables, export controls	Designed like technical review documents, with limited persistent navigation and high scanning cost
Organization and payer	/organizations, /payers	Public narrative pages	No working aggregate workspace exists yet

The current web application has several strong building blocks:

- The clinical caseload orders people by need and shows reasons instead of presenting an unexplained rank.
- Alerts show band, due time, overdue state, detail, audit trail, and a required documented resolution.
- The clinical person summary suppresses unsupported claims and shows evidence identifiers for rendered claims.
- Model-produced timeline entries are visually marked, and corrections append a superseding record.
- The interface states that ordinary overrides can relax pacing only, never a safety stop.
- The review gateway clearly labels fabricated data, purpose scope, and write-capable roles.

The next GUI should preserve those controls while removing duplication. `/clinician` and `/clinician/clinical` should become one workspace. `/clinician/member/[id]` and `/clinician/clinical/[id]` should become one person record with tabs or anchored sections. Governance consoles should sit under one Review and Controls area instead of appearing as equal top-level destinations beside daily clinical work.

3.3 Current iOS route and state inventory

The iOS application already contains far more than one dashboard. Its present screens fall into six groups.

Group	Current views and states	Current GUI pattern
Entry	Login, signup, local-only mode	Centered form and stacked cards
Program entry	Subscription, consent, fit screener, cooldown, five baseline instruments, seven-step profile	Long vertical forms and repeated soft cards
Main shell	Home, Check-in, Companion, Ground, Settings	Five-tab member shell with persistent SOS button
Daily support	Breathing, meditation, movement, sleep, learning	Repeated libraries of cards followed by simple players
Program	Module list, module detail, locked state	Ordered rows, tier badge, step list, single start action
Session	Intro, optional preparation, instruction, distress rating, bilateral sets, grounding pause, hard stop, completion, trigger capture	State machine with large single-purpose actions

3.4 Current member Home

The current Home screen stacks:

- Greeting
- Daily check-in card
- Optional Autopilot card

- Breathe card
- Meditate card
- Move card
- Sleep card
- Learn card
- Distress trend
- Twelve module rows
- Wellness disclaimer

This is a content catalog. It tells the member everything Steady can do, but it makes the member decide what matters now. On a hard day, that choice load is exactly what the system should reduce.

3.5 Current check-in result

The check-in correctly runs a deterministic decision. The result card gives a headline and detail. Except for a crisis result, it does not reliably carry the member into the recommended activity. The member must leave the result and find the right tool.

The result should become a decision card with:

- Today's state in plain language
- The main reason, without exposing sensitive scoring internals by default
- One recommended action
- One safe alternative
- How long the state applies
- A “why this?” drawer
- A crisis or support path that never depends on a successful save

3.6 Current progress display

The current chart shows ending distress across recent sessions. It lacks dates, baseline, goal or expected range, event annotations, confidence or missing-data handling, and a plain-language interpretation. “Lower is calmer” explains direction, but not meaning.

A trend should never stand alone. It should pair with:

- Comparison period
- Current value and change
- Plain-language interpretation
- Relevant event markers
- Missing or stale data notice
- A link to the underlying entries
- An accessible data table

3.7 Current gates and locks

Locked modules show “Not available right now” and a reason. This is a good base. The next version should distinguish:

- Temporarily limited by today's check-in
- Waiting for a named human review
- Locked by program sequence
- Unavailable because of consent or profile state
- Stopped by a safety rule
- Unavailable because data is stale or missing
- Unavailable because the service cannot confirm access

Those states are not interchangeable. They must not share one lock icon and one generic message.

3.8 Current safety language that must be corrected

The iOS hard-stop screen currently says a care team has been notified. The public demo says no one monitors the environment and no care team is assigned.

The GUI must never claim notification from an attempted write. Notification has its own state machine:

State	Member-safe language
Not configured	“This activity is not monitored. Use the support options below.”
Queued	“Steady is trying to send an alert. Do not wait for a response if you need help now.”
Delivered	“An alert was delivered to your care team at 2:14 PM. Response times vary.”
Acknowledged	“Jordan Lee acknowledged this alert at 2:19 PM.”
Failed	“Steady could not deliver the alert. Use the support options below now.”

Only a delivery receipt can support “delivered.” Only a separate acknowledgment event can support “acknowledged.”

3.9 Accessibility issue in the current color tokens

The main text colors perform well. Several accent colors do not meet the 4.5:1 contrast target for small text on ivory or linen.

Current token	Contrast on ivory	Use now	Required change
ground	10.54:1	Primary text	Keep
olive	6.03:1	Secondary text	Keep
support	5.24:1	Danger text	Keep, test every background
mistDeep	4.18:1	Lock and secondary accents	Do not use for small text on ivory
safeDeep	3.20:1	Success text	Replace for text
sageDeep	2.34:1	Links, labels, icons	Restrict to large non-text decoration or replace
clay	1.96:1	Selected backgrounds and accents	Background only

[W3C WCAG 2.2 contrast guidance](#) supports a 4.5:1 minimum for normal text. Focus indicators also need clear size and contrast, and controls should meet the WCAG 2.2 target-size rules. [W3C focus guidance](#) and [W3C target-size guidance](#) should be part of the release checklist.

4. What NeuroFlow and five comparable systems show

The products do not share one visual style. They do share a small set of information patterns that reduce the distance from data to action.

4.1 NeuroFlow

NeuroFlow uses different visual forms for the member, care team, and population viewer.

- The member experience centers tasks, activities, trackers, assessment prompts, progress, and rewards. Public app material lists Home, To Do, Discover, Crisis, and Profile as the primary navigation model.
- The tracker view lets a member switch among mood, sleep, stress, pain, and pain impact across selected time windows.
- Public provider images show a person summary with assignment completion, questionnaires, engagement, and alerts.
- Longitudinal measure screens use time-series charts, severity meaning, and suggested follow-up.
- Care-team dashboards support individual and population review, including at-risk cohorts and outliers.
- Journeys can be selected by the member, assigned by a care manager, or selected based on severity.

The main lesson for Steady is not gamification. It is role separation. The member sees manageable activities. The care team sees exceptions and follow-up. The enterprise viewer sees program performance.

Sources: [NeuroFlow digital behavioral health](#), [NeuroFlow solutions](#), [NeuroFlow measurement-based care](#), and [Onward by NeuroFlow on the App Store](#).

4.2 Blueprint

Blueprint's public dashboard image uses a strong two-column opening view.

- The left column is a notification queue. Each item names the person, the event, risk status, recency, and a Review action.
- The right column summarizes active, overdue, and high-risk clients.
- The product positions the dashboard as a way to get caught up before a session, not as a place to study raw records.
- The client app uses due assessments, daily check-ins, and personal trends.

The main lesson for Steady is that a clinician home should open on work, not charts. A chart belongs inside the person record or behind a population question.

Sources: [Blueprint guided measurement-based care](#), [Blueprint for Clients on the App Store](#).

4.3 Greenspace

Greenspace uses a restrained clinical dashboard with three visible levels.

- Top navigation separates Patients, Therapists, and Dashboard.
- The dashboard combines longitudinal symptoms and therapeutic alliance, number of patients, assessment completion, and a recent-activity heatmap.
- The patient list gives a progress snapshot before the user opens a detailed record.
- Organization views add adoption, engagement, and clinical-outcome benchmarks.

The main lesson for Steady is to keep population views small in number and stable in meaning. Adoption, engagement, outcomes, and activity are enough top-level groups. Each should drill into a defined cohort.

Sources: [Greenspace platform](#), [Greenspace benchmarks dashboard](#).

4.4 Osmind

Osmind uses an EHR-style shell organized around patient charts, schedule, and reports.

- A persistent navigation rail keeps high-frequency practice tasks available.
- Patient charts act as the center of the clinician workflow.
- The patient app extends the treatment plan into measures, journaling, messaging, medication reminders, and insurance documents.
- Public product material states that symptom-threshold alerts and automated rating scales sit within the charting workflow.

The main lesson for Steady is to attach between-visit data to the person's care context. A signal without the care plan, last review, ownership, and communication history creates more work.

Sources: [Osmind mental health EHR](#), [Osmind patient portal](#), [Osmind Care on the App Store](#).

4.5 Spring Health

Spring Health's current public materials show role-specific continuity.

- The provider home opens with today's sessions, risk tags, join or review actions, notes, invoices, and caseload tasks.
- The To Do list orders unfinished work by urgency.
- The person view brings anxiety, depression, sleep, clinical insights, notes, and session context into one continuous record.
- The member experience leads with the next step, such as booking another session.
- Employer views convert aggregate results into a small set of benchmarked insight cards.

The main lesson for Steady is to connect continuity with the next step. A member, provider, and organization can see different forms of the same underlying record without sharing the same dashboard.

Sources: [Spring Health Guide](#), [Spring Compass provider overview](#), [Spring Health member platform](#).

4.6 Eleos

Eleos focuses on work completion inside the provider's existing environment.

- The clinician flow is capture, draft, review, edit, and submit.
- Suggested note content is visibly reviewable before it becomes final.

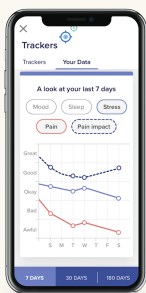
- Session analytics surface selected care and documentation signals rather than presenting a transcript as the main view.
- Leadership reporting highlights a few operating metrics, such as note volume, completion time, hours saved, activation, and active use.

The main lesson for Steady is that model output should enter a human task, not become a free-standing truth card. Suggested content needs a status, evidence, editable boundary, and final human action.

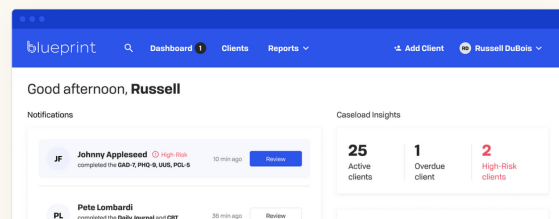
Sources: [Eleos documentation product](#), [Eleos community behavioral health](#).

5. Competitor screen evidence

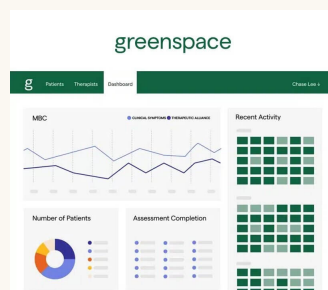
The screenshots below are public product images. They illustrate layout patterns, not verified end-to-end behavior.



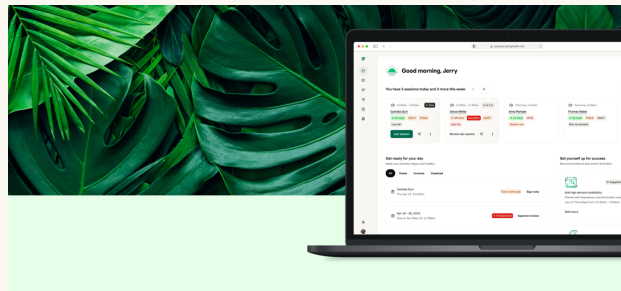
NeuroFlow member tracker
Capterra public product screenshot



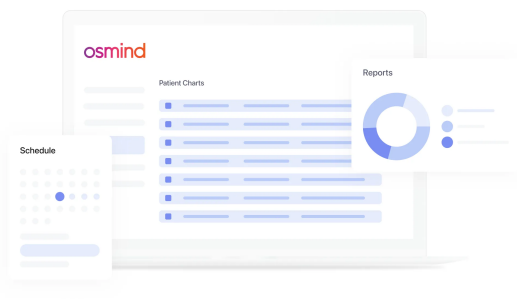
Blueprint clinician queue
Blueprint public product image



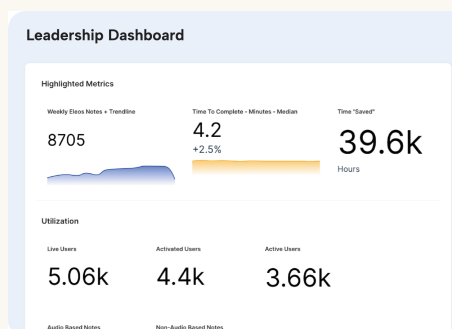
Greenspace aggregate dashboard
Qualifacts public product image



Spring provider day view
Spring Health public product image



Osmind chart, schedule, and reports
Osmind public product image



Eleos leadership dashboard
Eleos public product image

Images are included for comparative analysis. Product interfaces may have changed after the public image was published.

Pattern summary

Pattern	NeuroFlow	Blueprint	Greenspace	Osmind	Spring	Eleos
Member next step	Strong	Strong	Moderate	Care-plan led	Strong	Not primary
Clinician work queue	Strong	Strong	Patient-list led	Chart and schedule led	Strong	Documentation led
Individual trend	Strong	Strong	Strong	Strong	Strong	Session analytics
Aggregate dashboard	Strong	Limited public view	Strong	Reports	Strong	Strong operations view
Human review of model output	Limited public detail	Current AI workflow	Not primary	Current AI workflow	AI insights with oversight	Core pattern
Public GUI lesson for Steady	Separate roles	Put work first	Stable metric groups	Keep care context attached	Connect continuity to action	Make AI a review task

6. Target experience: one platform, different questions

Steady should not build one dashboard and hide columns by role. Each role has a different first question.

Role	First question	Primary object	First action
Member	“What is the safest useful thing for me now?”	Today plan	Start, choose another safe option, or get support
Clinician	“Who needs my attention and why?”	Work item	Review, contact, correct, approve, or defer with reason
Care manager	“What needs coordination today?”	Owned queue	Outreach, assign, close loop, or escalate
Supervisor	“Where is work backing up or varying?”	Team queue and operational metric	Reassign, coach, inspect, or change workflow
Organization leader	“Are people engaging and improving, and where are the gaps?”	Cohort metric	Drill down, compare, and plan an operating response
Payer or value-based lead	“Who is reached, what changes, and what does it cost?”	Defined population measure	Review performance and methodology
Security or compliance reviewer	“Who accessed or changed what, and can the record be trusted?”	Audit event and control state	Investigate, attest, export, or remediate

6.1 Shared product grammar

Every role should see the same six status concepts, expressed at the right level.

- 1
- State: What is true now.
- 2
- Change: What moved since the relevant comparison.
- 3
- Reason: What evidence and rule produced the state.
- 4
- Action: What this role is allowed to do.
- 5
- Owner: Who is responsible for the next step.
- 6
- Freshness: When the data and decision were last updated.

This grammar should live in API contracts and component props, not only in copy guidelines.

7. Target information architecture

7.1 Member navigation

Replace the current five tabs with:

Tab	Purpose	Current content moved here
Today	One daily plan, state, and recent completion	Check-in, Autopilot, immediate next action
Companion	Conversation and suggested actions	Companion, memory controls through profile
Practice	Browse safe activities and program modules	Breathe, meditate, move, sleep, Learn, modules
Progress	Understand change over time	Session trend, measures, milestones, timeline
Support	Immediate regulation and human help	Ground, SOS, crisis resources, safety plan

Profile and settings move to the avatar in the top-right corner. SOS remains reachable from every tab, but it should appear as a labeled “Support” control rather than a floating acronym alone. If the red floating control remains, its accessible and visible label should explain its function.

7.2 Clinician navigation

Route	Purpose
/clinical/work	Prioritized work queue
/clinical/people	Searchable caseload with compact state summaries
/clinical/people/[id]/overview	Current state, change, actions, care context
/clinical/people/[id]/timeline	Unified longitudinal timeline
/clinical/people/[id]/measures	Measures with meaning, ranges, and event markers
/clinical/people/[id]/plan	Goals, assignments, restrictions, and ownership
/clinical/reviews	AI summaries, gates, corrections, approvals, and overrides awaiting review
/clinical/reports	Caseload and workflow reporting
/clinical/audit	Role-scoped audit history

7.3 Organization navigation

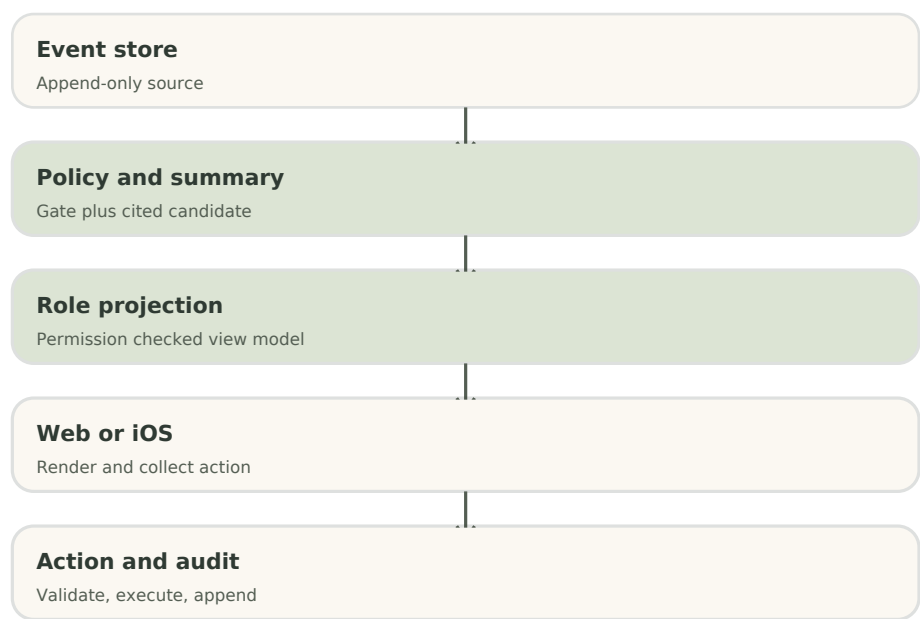
Route	Purpose
/organization/overview	Outcomes, engagement, operations, and access at a glance
/organization/engagement	Eligibility-to-active funnel, retention, completion
/organization/outcomes	Measure change, reliable improvement, stratified cohorts
/organization/operations	Queue time, outreach, review completion, workload
/organization/equity	Access and outcome differences with privacy controls
/organization/governance	Policy versions, review coverage, data quality, exports

7.4 Payer navigation

Route	Purpose
/payer/overview	Population reach, engagement, outcomes, and utilization
/payer/populations	Eligible, invited, enrolled, active, supported, improved
/payer/measurement	Definitions, denominators, time windows, exclusions
/payer/utilization	Approved cost and use measures when available
/payer/methodology	Cohort logic, attribution, missingness, and caveats

8. Presentation architecture

The UI should never build clinical meaning by joining raw event arrays in React or SwiftUI. The server should publish role-scoped presentation objects.



8.1 Layer responsibilities

Layer	Responsibility	Must not do
Event store	Preserve what happened	Decide what a user should see
Normalization	Convert events into typed signals	Apply role permissions late
Deterministic policy	Produce gates, bands, and allowed actions	Generate final user copy
Summary pipeline	Produce cited candidate statements	Bypass validation or human review state
Presentation service	Build role-scoped view models	Recalculate domain policy in the client
Client	Render, collect an authorized action, show state	Infer safety, fabricate freshness, or claim delivery
Action service	Validate and execute the action	Trust a client-provided owner, role, or gate
Audit service	Append actor, action, evidence, and result	Depend on display text as the source of truth

8.2 Shared presentation contract

```
type SourceKind = "event" | "measure" | "policy" | "review" | "integration";

interface EvidenceRef {
  id: string;
  kind: SourceKind;
  occurredAt: string;
  recordedAt: string;
  label: string;
  href?: string;
}

interface Freshness {
  computedAt: string;
  newestEvidenceAt: string;
  staleAfter: string;
  state: "fresh" | "aging" | "stale" | "unknown";
}

interface ReasonItem {
  code: string;
  label: string;
  detail?: string;
  evidence: EvidenceRef[];
}

interface AllowedAction {
  id: string;
  label: string;
  kind: "navigate" | "review" | "contact" | "approve" | "correct" | "override" | "export";
  href?: string;
  confirmation: "none" | "standard" | "high_risk";
  disabledReason?: string;
}

interface DecisionSurface {
  id: string;
  state: string;
  headline: string;
  explanation: string;
  change?: { label: string; direction: "better" | "worse" | "mixed" | "stable" | "unknown" };
  reasons: ReasonItem[];
  actions: AllowedAction[];
  owner?: { id: string; displayName: string; role: string };
  freshness: Freshness;
  policy?: { id: string; version: string };
  review?: { state: "not_needed" | "pending" | "approved" | "corrected" | "rejected";
    reviewedAt?: string };
}
```

The server creates the `DecisionSurface`. The clients render it. The clients may choose a mobile or desktop layout, but they may not change `state`, `reasons`, `allowed actions`, or `review state`.

8.3 Projection tables or caches

Create read-optimized projections so the screen does not assemble state from dozens of queries.

Projection	Key	Used by
member_today_projection	person, local date	Today screen
member_progress_projection	person, period	Progress screen
clinical_work_item_projection	organization, work item	Clinician and care-manager queues
person_status_projection	organization, person	Person header and caseload rows
person_timeline_projection	organization, person, cursor	Timeline

Projection	Key	Used by
gate_decision_projection	person, module, decision version	Member lock and clinician review
organization_metric_snapshot	organization, cohort, period	Organization views
payer_metric_snapshot	contract, population, period	Payer views
notification_delivery_projection	notification attempt	Member and staff notification copy

Each projection must include `computed_at`, `source_watermark`, `policy_version`, and `tenant_id`. A projection is disposable. The event and review records remain authoritative.

9. Gating as a visible, understandable state

9.1 Member-facing gate states

State	Visual treatment	Member message	Primary action
Open	Neutral or safe icon with text	“This is available today.”	Start
Caution	Warm neutral banner	“Go gently today because your activation is higher than usual.”	Start gentler option
Limited	Amber banner with reason	“Processing is paused today. Grounding and regulation remain open.”	Start grounding
Review needed	Blue-gray banner with owner and timing	“A specialist review is needed before this opens.”	See what happens next
Safety stop	High-contrast support banner	“This session stopped because continuing was not the safe choice.”	Ground now or get help
Unknown	Neutral warning	“Steady cannot confirm access right now.”	Use always-open tools and retry

Never rely on green, amber, or red alone. Every state needs an icon, a label, and one-sentence meaning.

9.2 Clinician-facing gate drawer

The clinician sees more detail without exposing raw implementation data in the first row.

Gate review

Person: Maya R. | Decision effective 10:42 AM

LIMITED

Processing paused today

Why

Activation rose; sleep low

Policy

daily-gate v14.3

Evidence

Check-in 10:41 AM | Measure 3 days ago | Prior gate Open

Allowed actions

Approve pacing | Correct reason | Contact

Review and record action

The drawer must show:

- Current decision and effective time
- Rule and version
- Human-readable reasons
- Evidence references
- What the clinician can approve, correct, or override
- What cannot be overridden
- Prior decision and change
- Member-safe copy preview
- Action confirmation and resulting audit record

9.3 Model-generated content

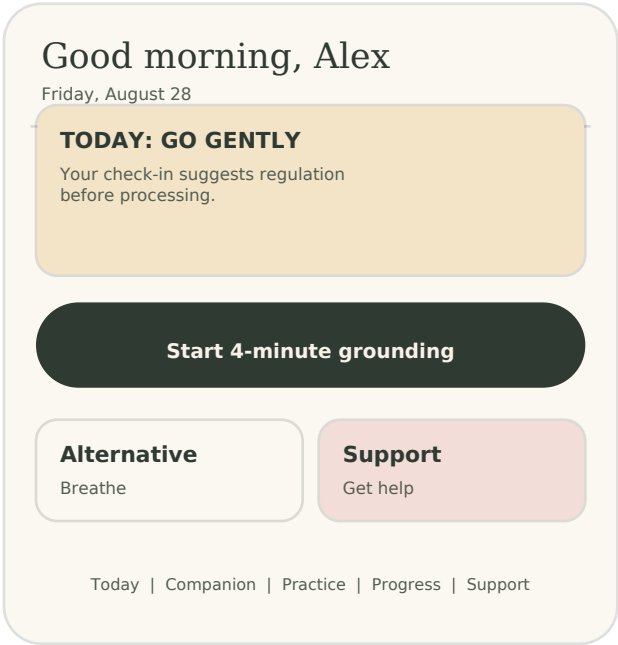
Use four distinct labels:

State	Label	Allowed placement
Candidate	"AI draft"	Review queue only
Validated	"Evidence checked"	Review queue and person view, still not approved
Human approved	"Reviewed by [name]"	Clinical record and downstream summary
Corrected	"Corrected by [name]"	Clinical record with superseded draft available in history

Do not use a sparkle icon as the only sign that text came from a model. Do not style model output like a system fact.

10. Screen specifications

10.1 Member Today



Purpose: remove choice load and connect the check-in to the next safe action.

Above the fold:

- Greeting and local date
- Data freshness only when stale or delayed
- Today state card
- One primary action with expected time
- One sentence explaining why
- Support control

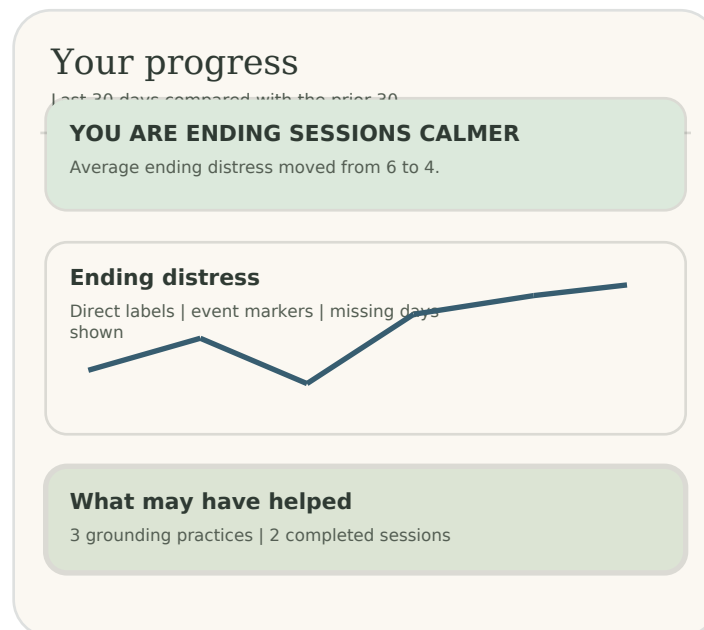
Below the fold:

- Up to two safe alternatives
- Completed activity and gentle progress
- Optional “why this plan?” drawer
- Upcoming care or review item if one exists

Rules:

- No full module catalog on Today.
- No paywall card above the member's safe action.
- If the check-in is due, it becomes the primary action.
- If a check-in returns Limited or Safety stop, the next action updates in place.
- If data is stale, say what is stale and preserve always-open options.

10.2 Member Progress

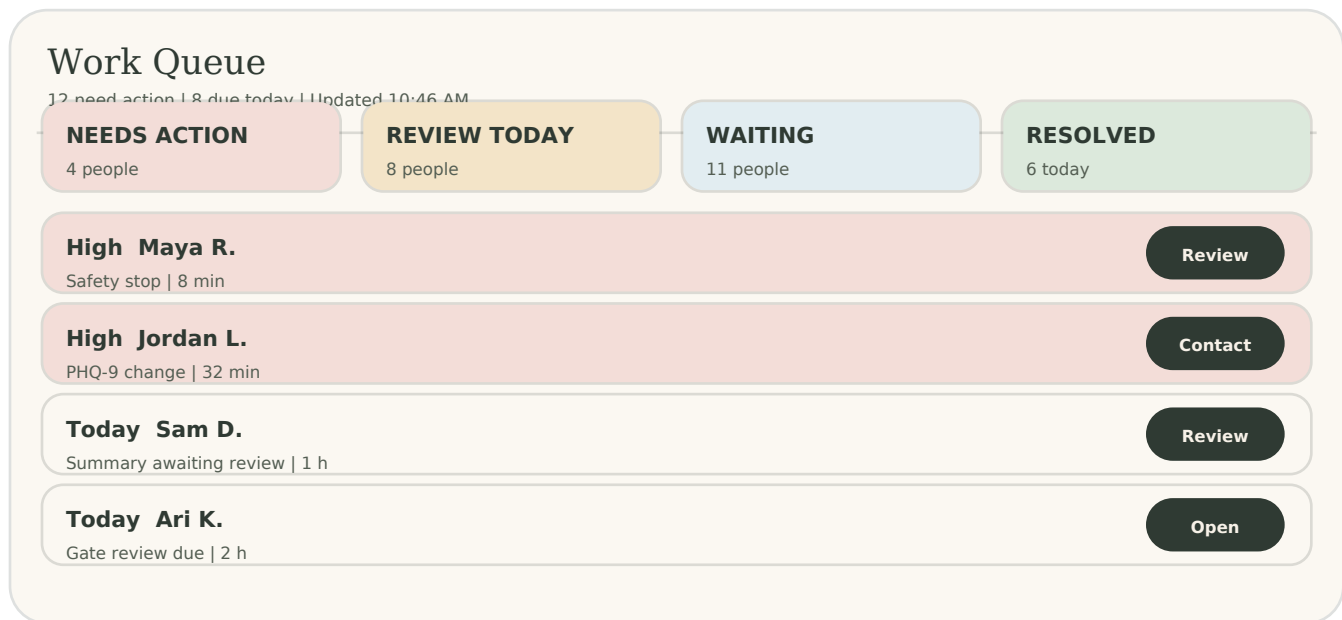


Opening order:

- 1 Plain-language change statement.
- 2 Current period compared with prior period.
- 3 One primary trend.
- 4 Event markers that may explain movement.
- 5 Measure cards and session history.
- 6 Full data table and source history.

Do not merge measures with different scales into one unlabeled line chart. If several signals matter, show small multiples with the same time range or a summary index whose construction is disclosed.

10.3 Clinician Work Queue



Required row fields:

- Priority band and label
- Person display identity
- Reason for appearing
- Change since last review
- Most recent evidence time
- Current owner
- Due or response target
- One row action

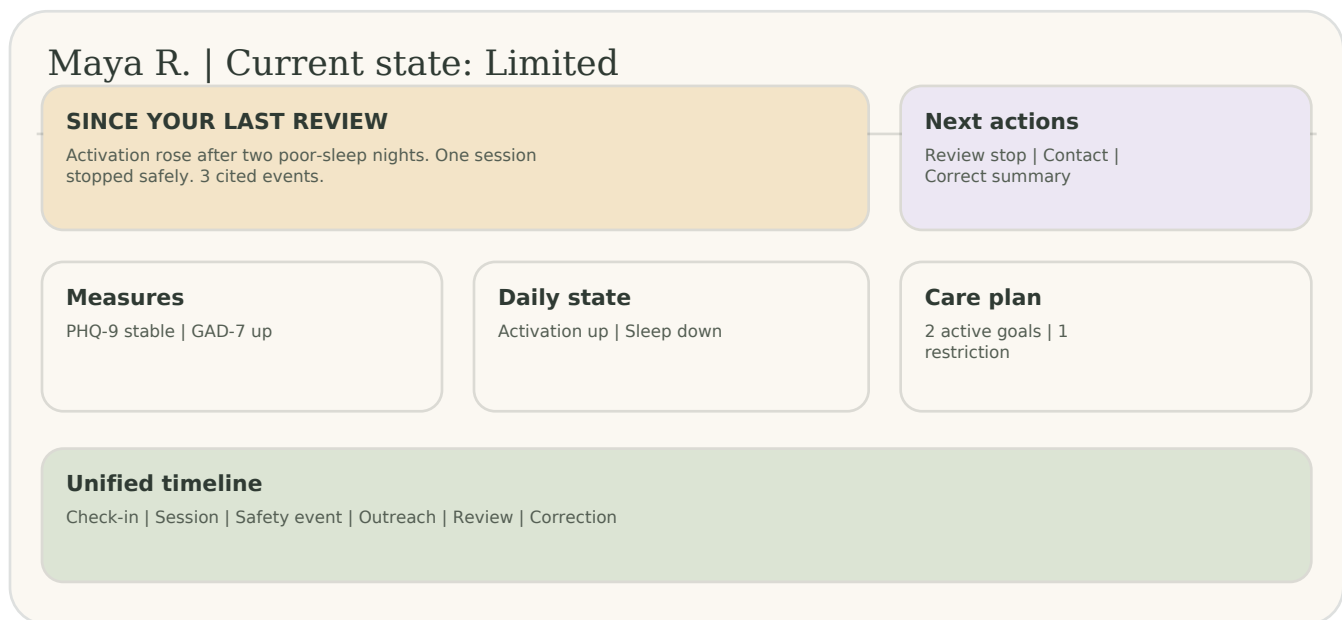
Queue groups:

- Needs action now
- Review today
- Waiting on member

- Waiting on staff or integration
- Recently resolved

Duplicate alerts for the same person and reason should collapse into one work item with an event count. AHRQ describes alert fatigue as desensitization caused by too many alerts, especially when most do not require action. [AHRQ alert fatigue primer](#). Steady should measure alert volume, duplicate suppression, time to action, and the share of alerts that result in a meaningful action.

10.4 Clinician Person Overview



Sticky header:

- Display identity
- Care relationship and owner
- Current state and freshness
- Consent or sharing boundary
- Contact action
- More menu for lower-frequency actions

Main content:

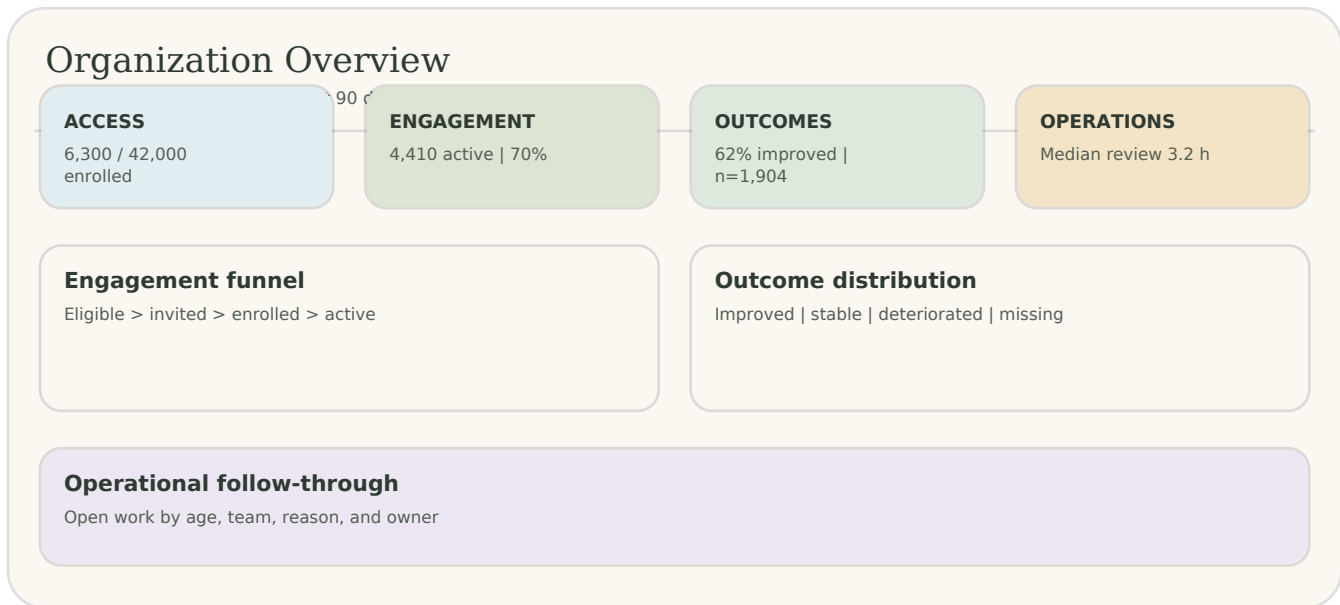
- “Since your last review” summary with citations
- Active work and pending reviews
- Measures and daily-state trends
- Current care plan, restrictions, and member preferences
- Unified timeline

Right rail:

- Allowed actions
- Safety plan access

- Assigned team
- Integration or sync warnings

10.5 Organization Overview

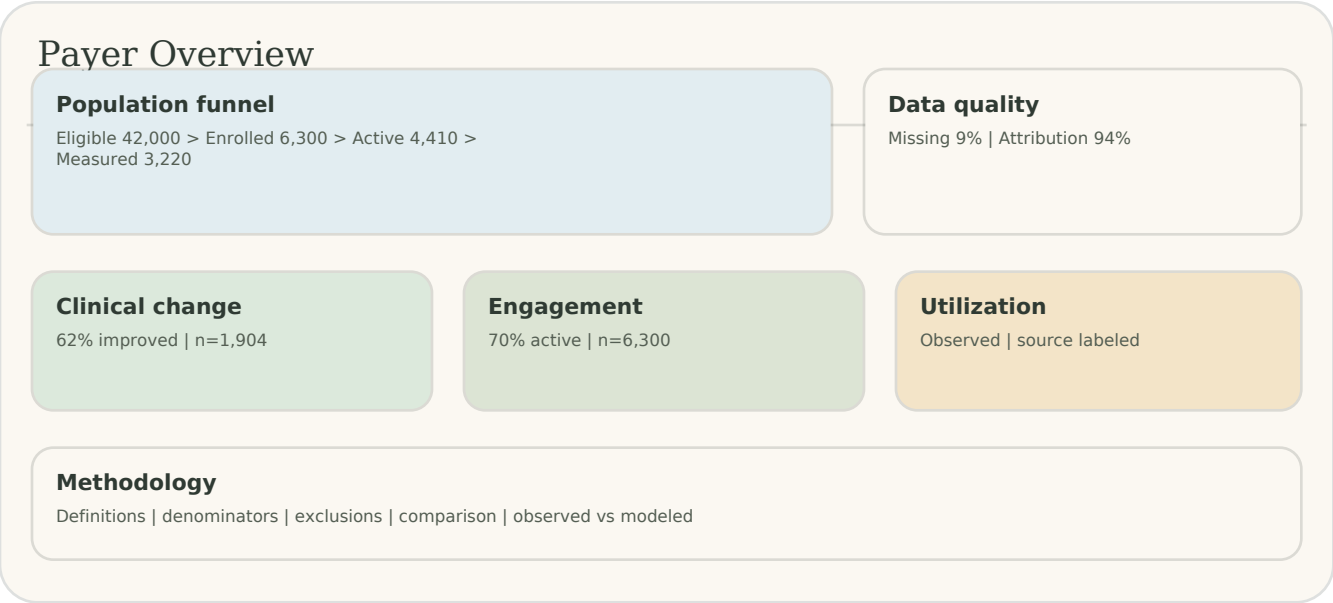


Use four stable sections:

- 1 Access: eligible, invited, enrolled.
- 2 Engagement: active, task completion, retention.
- 3 Outcomes: measure change, reliable improvement, deterioration.
- 4 Operations: queue time, reviews, outreach, unresolved work.

Every metric card must show numerator, denominator, time range, change, and cohort filter. A percentage without its denominator is not acceptable.

10.6 Payer Overview



The payer screen should not resemble a clinician dashboard. It should open with a population funnel and a small number of contract-defined measures.

Required disclosures:

- Eligible population definition
- Attribution rule
- Measurement window
- Minimum enrollment duration
- Missing-data rate
- Exclusions
- Comparison group, if any
- Whether a value is observed, modeled, or illustrative

No person-level drilldown should exist unless contract, permission, minimum-necessary purpose, and policy explicitly support it.

10.7 Trust and Audit

The trust workspace should convert the current long control page into a working review surface.

View	First information	Primary action
System health	Current control state and failures	Inspect failure
Audit integrity	Chain verification result and last check	Verify or export
Access review	Users, roles, last access, exceptions	Revoke or attest
Data flow	Current integrations and data classes	Inspect boundary
Policy versions	Active, pending, retired versions	Compare or approve

11. Component library

Build the following shared components before full page work.

Component	Job	Required props
DecisionCard	State, meaning, next action	state, headline, explanation, actions, freshness
ReasonList	Explain why	reason codes, labels, evidence links
GateStateBanner	Show access state	state, member copy, expiry, safe alternative
WorkQueueRow	Turn a signal into owned work	priority, person, reason, change, due, owner, action
ChangeStatement	Explain trend in words	current, comparison, direction, period, missingness
TrendPanel	Show a time series with meaning	series, range, bands, markers, table fallback
FreshnessLabel	Show age of data	computed time, newest evidence time, stale state
EvidenceDrawer	Show provenance	source references, policy, summary validation
ReviewBadge	Show human-review state	pending, approved, corrected, rejected
ModelDraftPanel	Hold model content safely	draft, citations, validator result, review actions
OwnerChip	Show responsibility	person or team, due state
MetricCard	Aggregate result with denominator	value, numerator, denominator, period, change
EmptyState	Explain why nothing appears	reason, expected next step, retry or support action
ServiceState	Explain loading, partial, stale, and failed states	service, state, fallback, last success

12. Visual tokens

Keep Steady's identity, but split brand colors from semantic text colors.

12.1 Base tokens

Token	Value	Use
surface.canvas	#F7F1E8	Main background
surface.card	#FBF8F2	Cards and panels
surface.subtle	#DCE4D4	Quiet grouped content
text.primary	#2F3A33	Main text
text.secondary	#545E53	Secondary text
border.default	rgba(47,58,51,.16)	Card and table boundary
brand.sage	#A8B8A1	Decorative fill and selected background
action.primary	#2F3A33	Primary button background
action.primaryText	#F7F1E8	Primary button text
focus.ring	#2F3A33	Visible keyboard focus

12.2 Semantic tokens

State	Text or icon	Background	Secondary sign
Safe or complete	#3F6A43	#DCE9DC	Check icon plus label
Information	#375D70	#E2EDF1	Info icon plus label
Caution	#7A5110	#F3E4C8	Triangle icon plus label
Support or stop	#8A4335	#F3DDD8	Stop or phone icon plus label
Pending review	#5B4B78	#ECE7F3	Clock icon plus label
Unknown	#545E53	#ECE9E2	Question icon plus label

Semantic text colors must pass 4.5:1 against their backgrounds. Chart colors need direct labels or distinct line styles. Color may reinforce meaning but cannot carry it alone.

12.3 Type and density

- Use serif display type for page identity, human explanation, and member-facing completion moments.
- Use sans-serif for controls, tables, labels, measures, and dense clinical work.
- Keep member body copy at 16 points or larger on mobile.
- Use a compact desktop density only in clinician, organization, payer, and audit tables.
- Use a 4-point spacing base with 8, 12, 16, 24, 32, and 48 as the working scale.
- Limit general card radius to 16 pixels on desktop and 20 pixels on mobile. Reserve the 24-pixel soft card for member emotional-support moments.

13. Charts and data explanation

Dashboard charts should use position and length for comparison whenever possible. Nielsen Norman Group notes that these encodings are faster to interpret than area and color alone. [NN/g dashboard chart guidance](#).

Required chart contract

Every chart must supply:

- Title written as a question or answer
- Plain-language interpretation
- Time range and timezone
- Current and comparison value
- Scale and unit
- Missing-data behavior
- Direct series labels
- Event annotations when useful
- Accessible table
- Download or source link for authorized users

Default chart choices

Question	Default chart
How has one measure changed?	Line chart with severity bands and event markers
Which work group is largest?	Horizontal bar chart
Where are people in a funnel?	Ordered step bars with counts and conversion
Has a cohort improved?	Distribution or reliable-change bars, not only an average
Is work backing up?	Aging buckets and median time, not a decorative gauge
Did an individual action happen?	Timeline event, not a chart

Avoid donut charts when users must compare close values. Avoid more than three lines in a default view. Avoid dual axes. Avoid red-to-green scales without labels and patterns.

14. Empty, stale, partial, and failed states

Health software cannot treat “no data” as one condition.

State	Example	Required UI behavior
True empty	No sessions yet	Explain what will appear after the first session
Not due	Assessment not scheduled	Show next due date, not zero
Missing	Expected measure not completed	Show missing state and allowed follow-up
Delayed	Integration data behind	Show last successful update and do not infer current state
Partial	Some services loaded	Render safe sections and name what is unavailable
Unauthorized	Role cannot view data	Do not reveal that a hidden record exists
Failed	Projection cannot load	Show retry and safe fallback; log correlation ID privately
Stale decision	Gate outlived its valid window	Re-run gate or limit activity according to policy

The public issue page pattern of turning database failure into a legitimate empty result must not appear in Steady's clinical views. Empty and error are different states.

15. Action and audit behavior

15.1 No optimistic updates for high-impact actions

Approve, correct, override, contact confirmation, safety review, and notification delivery must wait for server confirmation. Low-risk actions such as expanding a drawer or changing a local filter may update immediately.

15.2 Confirmations

Action	Confirmation
Approve cited summary	Standard confirmation with content hash
Correct summary	Show final text, reason, and superseded version
Override pacing rule	High-risk confirmation with scope and expiry
Attempt safety-stop override	Do not render the action
Mark outreach complete	Require channel, result, and timestamp
Export person-level data	Require purpose, scope, and audit entry

15.3 Action response

Every mutation response should return:

- New presentation object
- Audit event ID
- Effective time
- Result state
- Any follow-up work item
- Any failed downstream delivery state

The client should render the returned object. It should not patch individual fields based on what it hoped happened.

16. Suggested endpoints

```

GET /api/v1/member/today
POST /api/v1/member/check-ins
GET /api/v1/member/progress?period=30d
GET /api/v1/member/practices?availability=today

GET /api/v1/clinical/work-items
GET /api/v1/clinical/people
GET /api/v1/clinical/people/:id/overview
GET /api/v1/clinical/people/:id/timeline
GET /api/v1/clinical/people/:id/measures
POST /api/v1/clinical/reviews/:id/approve
POST /api/v1/clinical/reviews/:id/correct
POST /api/v1/clinical/gates/:id/override

GET /api/v1/organization/overview
GET /api/v1/organization/outcomes
GET /api/v1/organization/operations
GET /api/v1/payer/overview
GET /api/v1/payer/methodology
GET /api/v1/audit/events

```

All responses are tenant scoped on the server. The client never supplies a trusted tenant ID or role. Cursor pagination is required for work items, people, timeline, and audit events.

17. Web and iOS rendering boundary

Web

- Server components may load the initial role projection.
- Client components manage filters, drawers, chart range, and action dialogs.
- Route-level error boundaries must distinguish unavailable service, unauthorized access, and true empty results.
- Tables need keyboard navigation, visible focus, row actions, and responsive card fallbacks.

iOS

- Map shared JSON contracts into immutable Swift view models.
- Keep local deterministic safety checks as an offline safety floor.
- Never invent a server delivery or review state while offline.
- Today may show a locally safe fallback plan when disconnected, labeled “On this device.”
- Use Dynamic Type, VoiceOver labels, reduced-motion behavior, and non-drag alternatives for every slider-driven safety input.
- Preserve the session state machine. Redesign its framing and completion summary without merging or skipping safety states.

18. Implementation sequence

Phase 0: Presentation contract

- Add shared `DecisionSurface`, evidence, freshness, action, and review types.
- Add notification delivery states.
- Add server-side role projection services.
- Add fixture builders for every status and failure state.
- Freeze direct raw-event consumption in new UI code.

Exit: member Today and clinician Work Queue can be rendered entirely from fixtures with no client-side domain calculations.

Phase 1: Tokens and components

- Replace low-contrast semantic text tokens.
- Build `DecisionCard`, `GateStateBanner`, `WorkQueueRow`, `FreshnessLabel`, `EvidenceDrawer`, `TrendPanel`, `ReviewBadge`, and service states.
- Add Storybook or an equivalent component gallery on web.
- Add SwiftUI previews for all member states.

Exit: component accessibility tests and screenshot baselines pass.

Phase 2: Member shell

- Build Today, Practice, Progress, Support, and profile navigation.
- Connect check-in result directly to the safe next action.
- Move the catalog out of Today.

- Add plain-language progress summaries and chart tables.
- Correct the notification language state machine.

Exit: a member can complete the main daily path without browsing the catalog or interpreting a raw score.

Phase 3: Clinical cockpit

- Build Work Queue and caseload.
- Build Person Overview, measures, timeline, plan, and review drawer.
- Add ownership, due state, duplicate collapse, freshness, and citation access.
- Implement approve, correct, and allowed override actions with full audit response.

Exit: a clinician can identify the next person, understand why, act, and verify what was recorded from one flow.

Phase 4: Organization, payer, and trust workspaces

- Build stable aggregate metric groups.
- Add numerator, denominator, cohort, time, and missingness to every metric.
- Add role-specific exports.
- Build system health, access review, policy, and audit views.

Exit: each role can answer its first question without entering another role's workspace.

Phase 5: Human-factors validation

- Run role-based usability sessions with fabricated scenarios.
- Measure time to first correct action, error rate, interpretation accuracy, and confidence.
- Run accessibility review with keyboard, screen reader, zoom, Dynamic Type, reduced motion, and color-vision simulation.
- Tune alert thresholds and duplicate suppression from pilot data under governance.

Exit: release criteria below are met with documented evidence.

19. Engineering backlog

Epic	Required output
GUI-01 Presentation contracts	Shared schemas, validators, fixture factory, versioning
GUI-02 Role projections	Member, clinical, organization, payer, audit read models
GUI-03 Design tokens	Accessible semantic palette, type, spacing, focus, chart tokens
GUI-04 Shared components	Decision, gate, queue, trend, evidence, review, state components
GUI-05 Member Today	Check-in-to-action flow, fallback, alternatives, support
GUI-06 Member Progress	Interpretation, trends, markers, table, source history
GUI-07 Clinical Work	Queue groups, ownership, due state, duplicate collapse
GUI-08 Person Record	Overview, measures, timeline, plan, actions, right rail
GUI-09 Review actions	AI draft, validation, approval, correction, allowed override
GUI-10 Aggregate views	Organization and payer metrics with definitions
GUI-11 Trust workspace	System state, audit, access review, data flow, policy versions
GUI-12 Quality	Accessibility, visual regression, permissions, failure states, telemetry

20. Acceptance criteria

20.1 Universal

- Every state card answers state, reason, action, and freshness.
- Every action shown is authorized for that role and current state.
- No client component recalculates safety or priority.
- Empty, stale, partial, failed, and unauthorized states render differently.
- Status never relies on color alone.
- Normal text meets WCAG AA contrast.
- Keyboard focus remains visible and unobscured.
- Charts have accessible tables and direct series labels.
- No analytics event contains free text, measure answers, or person identifiers.

20.2 Member

- Today shows no more than one primary and two secondary actions.
- The check-in result changes the next action without requiring navigation to a catalog.
- Every limited or stopped state gives a safe alternative.
- Support remains reachable during offline, write failure, and service failure states.
- The app never says a care team was notified without a delivered event.
- The member can view and change companion memory controls.

20.3 Clinician and care manager

- Queue order is stable for the same policy version and evidence set.
- Each work item states why it exists and when the evidence arrived.
- Duplicate events collapse without losing count or evidence.
- Approve, correct, and override remain separate actions and audit types.
- Safety-stop override does not appear.
- Person summary claims open their cited evidence.
- Cross-tenant and unauthorized person requests return no record detail.

20.4 Organization and payer

- Every rate includes numerator and denominator.
- Every metric includes time range and cohort definition.
- Missingness and exclusions are visible.
- Small cohorts follow the approved suppression rule.
- Individual drilldown is absent unless explicitly authorized.
- Export content matches the on-screen filters and records them in audit history.

21. Test matrix

Test family	Required cases
Contract	Schema version, unknown state, missing evidence, stale data, action permission
Gate presentation	Open, caution, limited, review, stop, unknown, expiry, offline
Notification truth	Not configured, queued, delivered, acknowledged, failed
Queue	Order, duplicate collapse, owner, due time, reassignment, resolved state
Person record	New event, corrected summary, retired policy, delayed integration
Aggregate	Denominator, missingness, cohort filter, small-cell suppression
Permission	Member, clinician, manager, supervisor, organization, payer, security
Accessibility	Keyboard, VoiceOver, screen reader, zoom, Dynamic Type, reduced motion
Visual regression	All semantic states at mobile, tablet, laptop, and wide desktop
Failure	Projection error, timeout, partial response, stale cache, write conflict
Audit	Action ID, actor, role, evidence, old state, new state, result

22. Product telemetry

Measure whether the GUI reduces work, not only whether it receives clicks.

Member measures

- Time from opening Today to starting the primary action

- Check-in completion to recommended-action start
- Abandonment by screen and state
- Use of safe alternatives
- Support access during failure states
- Progress-interpretation accuracy in usability testing

Clinician measures

- Time to first meaningful action
- Work-item aging by priority
- Duplicate alerts suppressed
- Alert-to-action rate
- Summary approval, correction, and rejection rates
- Evidence-drawer use before high-impact actions
- Queue items transferred or returned because ownership was unclear

Organization measures

- Time to answer a defined program question
- Metric-definition opens
- Export frequency and purpose
- Difference between displayed and exported cohort counts

Telemetry must use coded event types and pseudonymous IDs. It must not record member free text, message content, trigger names, measure answers, or clinical notes.

23. Final engineering rules

- 1 Do not make the client infer domain meaning.
- 2 Do not show an alert without a clear owner and possible action.
- 3 Do not show a percentage without its denominator.
- 4 Do not show a gate without a reason and a safe alternative.
- 5 Do not show model text without its status and evidence.
- 6 Do not claim delivery from an attempted write.
- 7 Do not use the same home screen for every role.
- 8 Do not hide stale or missing data behind a clean chart.
- 9 Do not let visual polish weaken Steady's safety or audit boundaries.
- 10 Make the shortest path through every screen match the user's actual job.

The intended result is not a prettier data system. It is a system that helps a member choose the next safe step, helps a clinician find and complete the next needed review, and helps an organization understand whether care is reaching people and changing outcomes.

Source notes

- [Current Steady public environment](#), inspected August 28, 2026.
- [Current Steady web repository](#), commit c39447a715e92b4800889c7afd6d015e39457c8f.
- [Current Steady iOS repository](#), commit f9a2645fc57135dfe1c7ee61c0e327a4ddc35a0d.
- [NeuroFlow digital behavioral health](#).
- [NeuroFlow solutions](#).
- [NeuroFlow measurement-based care](#).
- [Onward by NeuroFlow on the App Store](#).
- [Blueprint guided measurement-based care](#).
- [Blueprint for Clients on the App Store](#).
- [Greenspace platform](#).
- [Greenspace benchmarks dashboard](#).
- [Osmind mental health EHR](#).
- [Osmind patient portal](#).
- [Osmind Care on the App Store](#).
- [Spring Health Guide](#).
- [Spring Compass provider overview](#).
- [Eleos documentation product](#).
- [AHRQ alert fatigue primer](#).
- [HealthIT.gov clinical decision support](#).
- [W3C WCAG 2.2 reference](#).
- [NN/g dashboard chart guidance](#).